SACTCHECK · REGIMEN-SPECIFIC SACT CONSENT

PROTOTYPE · GOVERNANCE REVIEW REQUIRED

Pembrolizumab · every 3 weeks · advanced melanoma

Unresectable or metastatic melanoma in adults; the team confirms the source-specific eligibility and previous treatment. · NCCP 00455 · version 16 · 21-day cycle

Patient name	DOB	Hospital no.	Date

Treatment context
Pembrolizumab helps immune cells act against melanoma that cannot be completely removed or has spread. The aim is disease control; benefit and side effects are reviewed for you.

Schedule
Pembrolizumab every 3 weeks. Treatment continues until progression or unacceptable toxicity. For sustained response without progression, stopping after 24 months may be considered by the treating team; this is not an automatic stop.

1. TREATMENT DISCUSSION

- ☐ Purpose, expected benefit and alternatives explained
- ☐ Indication, medicines, sequence and duration confirmed
- ☐ Blood tests and any individually indicated heart checks discussed
- ☐ Support medicines and phase changes explained
- ☐ Fertility, contraception and breastfeeding discussed where relevant
- ☐ Common, serious and lasting drug-specific risks discussed below
- ☐ Urgent symptoms and local contact details confirmed

2. REGIMEN / AGENT-SPECIFIC MATERIAL RISKS

Pembrolizumab
PD-1 CHECKPOINT IMMUNOTHERAPY

- ☐ Immune inflammation of bowel, lungs, liver or kidneys
- ☐ Thyroid, pituitary, adrenal and glucose problems
- ☐ Skin reactions and rare severe skin injury
- ☐ Rare heart, muscle, neurological and eye inflammation
- ☐ Infusion reactions
- ☐ Some immune effects need long-term treatment
- ☐ Heart, muscle and nerve immune effects can occur together and require urgent assessment
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Discuss the exact diagnosis, treatment intent, expected benefits and uncertainty, suitable alternatives, the regimen-specific risks and monitoring, fertility and pregnancy precautions, and practical burdens. Confirm the treatment phase and route. This guide supports the conversation and does not replace individual consent. Discuss the possibility that heart, muscle and nerve inflammation can occur together and may need urgent hospital treatment. A severe immune reaction may mean treatment must remain stopped even after recovery. Discuss the possibility that heart, muscle and nerve inflammation can occur together and may need urgent hospital treatment. A severe immune reaction may mean treatment must remain stopped even after recovery.

Individual aim / benefit, alternatives, material risks and questions:

3. CONSENT CONFIRMATION

- ☐ Expected benefit, important risks and reasonable alternatives discussed
- ☐ Patient questions invited and answered; patient-specific material risks / priorities considered
- ☐ Emergency contact / toxicity advice and written information offered where appropriate
- ☐ Patient understands consent is voluntary and may be withdrawn

Patient / person giving consent _____ Date _____

Clinician obtaining consent _____ Date _____

Prototype only. This one-page form is a regimen-specific consent aid and requires formal governance, consultant and oncology-pharmacy review before clinical use. The current NCCP regimen, local/national consent policy, current medicine information, patient-specific material risks and clinical judgement remain authoritative.
Base regimen: NCCP 00455 v16.

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